

Women's Specialists of Clear Lake

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Big Baby? Should I Be Induced at 38 Weeks?

New high-quality evidence (Lancet 2025, AJOG-MFM 2026) supports offering induction at 38+0 to 38+4 weeks when baby's estimated weight is >90th percentile

WHAT 'LARGE FOR GESTATIONAL AGE' MEANS

- **LGA** = estimated fetal weight (EFW) **above the 90th percentile** for gestational age, measured by ultrasound
- **Macrosomia** = birth weight **≥ 4,000 g (about 8 lb 13 oz)**, or **≥ 4,500 g (9 lb 15 oz)** for severe macrosomia
- Ultrasound estimates of weight are **± 10-15% accurate** -- they are a guide, not a guarantee
- Most LGA babies are healthy and deliver without complications
- Some risks rise with larger babies: **shoulder dystocia, fracture during birth, C-section, perineal injury**, and (for baby) low blood sugar or jaundice
- Risk is higher if you have **diabetes** -- those patients are managed separately

WHAT THE NEW RESEARCH SHOWS

- **Big Baby Trial (UK, Lancet 2025)** -- 2,893 women with suspected LGA, induced at 38+0 to 38+4 vs. standard care
- **Shoulder dystocia** -- 2.3% (induction) vs. 3.1% (standard); in the per-protocol analysis, induction cut the risk **almost in half** (2.3% vs 3.7%)
- **Meta-analysis (AJOG-MFM 2026, 5 trials, ~4,000 women)** -- induction at 38 wks **reduced cesarean delivery** (27.9% vs 31.9%)
- Induction also **dramatically reduced very large birth weights** (≥ 4,000 g cut nearly in half; ≥ 4,500 g cut by ~80%)
- **Number needed to treat: about 25 inductions to prevent 1 cesarean**
- Trend toward fewer **birth fractures** and fewer **shoulder dystocia events**, though not always statistically significant

WHO THIS APPLIES TO

- ✓ Single baby, head down, at term
- ✓ **Estimated fetal weight > 90th percentile** on an ultrasound between 35-38 weeks
- ✓ **Healthy pregnancy** with no other reason for early delivery
- ✓ No major contraindications to vaginal birth or induction
- ✓ **Not included in the research:** patients with **gestational or pre-existing diabetes** -- those are managed by separate guidelines
- ✓ Patients already planned for cesarean or with another induction indication

POTENTIAL BENEFITS OF INDUCING AT 38 WEEKS

- ✓ **Lower chance of cesarean delivery** (about 4 fewer C-sections per 100 women)
- ✓ Much **lower chance of a very large baby** (>4,000 or >4,500 g)
- ✓ Likely lower risk of **shoulder dystocia** and the injuries that can come with it
- ✓ **Less time waiting and worrying** about size and timing
- ✓ Babies born at 38 weeks are **mature** and do well in nearly all cases
- ✓ More predictable scheduling for you and the delivery team

POTENTIAL DOWNSIDES

- **More newborn jaundice** requiring phototherapy (treatable light therapy) -- the main difference seen in the meta-analysis
- Induction takes time -- can be a **longer hospital stay** before delivery
- Some women find induced labor more uncomfortable; **epidural is available**
- Ultrasound weight estimates can be **off** -- baby may be smaller than predicted
- Induction does not guarantee a vaginal birth; **cesarean is still possible**
- Earlier birth means slightly less time for some final development (rarely an issue at 38+ weeks)

MAKING THE DECISION TOGETHER

- ✓ This is a **shared decision** -- there is no one-size-fits-all answer
- ✓ We will review your **specific ultrasound numbers, history, and goals**
- ✓ **Best evidence supports the 38+0 to 38+4 week window** if you choose induction
- ✓ **Standard care (wait for spontaneous labor)** is also reasonable, especially if EFW is closer to the 90th percentile
- ✓ Tell us if you have a **strong preference** for vaginal birth, an earlier or later delivery, or avoiding induction
- ✓ Ask about **cervical ripeness** (Bishop score) -- a favorable cervix makes induction more likely to succeed
- ✓ Bring your **birth partner and questions** to the next visit so we can plan together

⚠️ CALL OR GO TO LABOR & DELIVERY IF YOU EXPERIENCE

- ⚠️ Regular, painful contractions (call us)
- ⚠️ Severe abdominal pain
- ⚠️ Your water breaks
- ⚠️ Severe headache, vision changes, or upper-belly pain

⚠ Vaginal bleeding more than spotting

⚠ Decreased baby movement

⚠ Sudden swelling of hands, face, or one leg

⚠ Any concern that something feels wrong -- trust your instincts

Good Evidence, Real Choices

Recent high-quality research (the UK Big Baby trial and a 2026 meta-analysis of about 4,000 women) shows that offering induction between 38+0 and 38+4 weeks for babies estimated above the 90th percentile can lower the chance of cesarean delivery and very large birth weight, with a likely reduction in shoulder dystocia. The main trade-off is slightly more newborn jaundice. Both early induction and waiting for spontaneous labor remain reasonable options -- the right choice depends on your numbers, your history, and what matters most to you. Bring your questions; we'll decide together.